DRUGS · VERTIGO AND MÉNIÈRE'S DISEASE

Diuretics in Ménière's disease

Diuretics have been used in Ménière's disease for decades on the theory that they reduce endolymphatic hydrops. Thiazides (bendroflumethiazide, hydrochlorothiazide, often with a potassium-sparing drug) and acetazolamide are the agents. Evidence of benefit is weak and comes mostly from older, small studies, but many patients and clinicians report fewer attacks. They need electrolyte monitoring.

1 CHAPTER ONE At a glance

The drugs on one page.

1.1 Summary

DIURETICS IN MÉNIÈRE'S DISEASE

	SUMMARY
Agents	Bendroflumethiazide; hydrochlorothiazide with triamterene or amiloride; acetazolamide (carbonic anhydrase inhibitor)
How they work	Thiazides block the Na-Cl cotransporter in the distal tubule; acetazolamide reduces bicarbonate reabsorption and may reduce endolymph production; both proposed to reduce hydrops
ENT uses	Maintenance treatment in Ménière's disease to reduce vertigo attacks (alongside salt restriction); acetazolamide also in idiopathic intracranial hypertension with pulsatile tinnitus, and in some CSF leaks
Adult dose	Bendroflumethiazide 2.5 mg once daily; hydrochlorothiazide 25 mg with triamterene once daily; acetazolamide 250 mg twice daily (higher in intracranial hypertension)
Avoid	Hypokalaemia, hyponatraemia, gout, severe renal or hepatic impairment; acetazolamide in sulfonamide allergy, renal stones, metabolic acidosis

2 CHAPTER TWO Using them in ENT

Indications.

2.1 Ménière's disease

Offered after lifestyle measures, often with or instead of betahistine; review at 3 to 6 months. Check sodium, potassium and renal function before and periodically. Salt restriction (about 1.5 to 2 g sodium a day) is part of the same strategy.

2.2 Pulsatile tinnitus and intracranial hypertension

Acetazolamide lowers CSF production in **idiopathic intracranial hypertension**, which presents with pulsatile tinnitus, headache, obesity and papilloedema; involve neurology and ophthalmology.

3 CHAPTER THREE Safety

What goes wrong.

3.1 Adverse effects

- Thiazides: hypokalaemia, hyponatraemia, hyperuricaemia and gout, hyperglycaemia, postural hypotension, erectile dysfunction, photosensitivity.
- Acetazolamide: paraesthesia, taste change (carbonated drinks taste flat), fatigue, metabolic acidosis, renal stones, rarely blood dyscrasias and Stevens-Johnson syndrome.

3.2 Interactions

Lithium (toxicity), NSAIDs (reduced effect), digoxin (toxicity if hypokalaemic).

4 CHAPTER FOUR For the examination

What the examiner wants to hear.

4.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- The rationale and the weak evidence.
- Monitoring of electrolytes.
- Acetazolamide in **idiopathic intracranial hypertension** with pulsatile tinnitus.

4.2 Common pitfalls

COMMON PITFALLS

- No electrolyte checks in an elderly patient on a thiazide.

Written for this app from BNF and BNF for Children; NICE and specialist society guidance; Scott-Brown; Cummings. Figures reproduced from the books for private study. Study reference only: follow local protocols and senior advice for patients.